INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Regimen source
NCCP 00776
Version 3b

21-day cycle

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Aim to control cancer growth, reduce cancer-related symptoms and prolong disease control. Individual benefit and duration vary; trial averages cannot predict one person's outcome.

HER2-DIRECTED ANTIBODY–DRUG CONJUGATE

Trastuzumab deruxtecan (Enhertu)

Uses HER2 as a docking target to deliver a topoisomerase-I inhibitor payload

Antibody attaches to HER2 → Cancer cell takes the linked drug inside → Released payload damages deoxyribonucleic acid (the cell’s genetic material) (DNA) during cell division

Enhertu links an antibody that recognises HER2 with deruxtecan, a cell-killing medicine. After uptake into the cell, the payload inhibits topoisomerase I and interferes with DNA replication. The released payload can also affect nearby tumour cells. This targeting does not spare every healthy cell, which is why blood counts, nausea, lungs and heart function still need attention.

New cough, breathlessness or fever needs urgent contact even between cycles. Keep the 12-weekly heart-scan plan and use the anti-sickness medicines as prescribed.

The exact treatment being discussed

Who this covers

NCCP00776a: unresectable/metastatic HER2-positive breast cancer after at least one anti-HER2 regimen. NCCP00776b: unresectable/metastatic HER2-low breast cancer after chemotherapy for metastatic disease, or recurrence during or within six months after adjuvant chemotherapy.

NCCP 00776 v3b · official regimen · EMA Enhertu product information · retrieved 25 October 2026

Dose and administration

Usual starting dose 5.4 mg/kg intravenously on Day 1 every 21 days; prescribed reductions are 4.4 then 3.2 mg/kg. The first infusion takes 90 minutes. Later infusions may take 30 minutes if the previous infusion was tolerated. Allow extra time for assessment, anti-sickness medicines and any observation. A cannula or an existing central line can be used according to the clinical plan; there is no home pump.

Duration and decisions

Treatment continues while it controls the cancer and side effects remain acceptable. This advanced-disease regimen has no automatic one-year completion date. A good scan does not by itself instruct a treatment break.

Individual safety and source differences

Enhertu is different from trastuzumab and trastuzumab emtansine (Kadcyla); they are not interchangeable. Carry the supplied Enhertu lung-safety alert card. The team may pause treatment while investigating suspected lung inflammation; symptomatic drug-related interstitial lung disease (ILD) requires permanent discontinuation under the protocol.

Evidence for HER2-positive and HER2-low advanced breast cancer

This explains the study setting and its meaning. Ask the team how the evidence applies to you.

Randomised open-label trial in a selected prior-treatment population. This PFS estimate is investigator assessed and must not be mislabeled as the original primary-endpoint analysis. Population medians are not individual forecasts. All-grade ILD/pneumonitis was 16.7% with T-DXd in this longer follow-up.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team urgently for any new or worsening cough, breathlessness or fever: lung inflammation can start at any point and can become serious. Do not wait until the next infusion or scan. Also report fever or feeling acutely unwell, unusual bleeding, inability to keep fluids down, chest pain or new ankle swelling. Severe breathing difficulty or collapse needs emergency care.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Blood count, kidney and liver tests	Before first dose	Before cycle 1	Before every 21-day cycle	Before every cycle; extra tests for symptoms/ results	Repeat after treatment holds or toxicity as directed
Echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline; eligibility left ventricular ejection fraction (heart pumping measurement) (LVEF) ≥50%	Use baseline result to confirm readiness	Week 12 ≈ before cycle 5 if uninterrupted	Every 12 weeks: week 24 ≈ cycle 9; 4–8 weeks if impairment	New threshold-level LVEF decline: repeat within 3 weeks; cardiology plan
Lung symptoms / computed tomography (CT) if suspected interstitial lung disease (ILD)	Review prior ILD and baseline symptoms	Explain alert card and urgent contact	Ask at each contact; no symptom-free guaranteed window	New cough, breathlessness or fever → prompt assessment/ CT	Do not rechallenge after symptomatic drug-related ILD
Pregnancy / reproductive care	Pregnancy test where relevant; fertility and contraception discussion	Continue contraception	Repeat pregnancy testing if clinically indicated	Continue agreed precautions	Contraception after final dose: women ≥7 months; men ≥4 months
dual-energy X-ray absorptiometry (bone density) (DEXA) / tumour markers	Only if another treatment or individual indication warrants them	No automatic Enhertu-specific schedule	Use only if clinically indicated	Do not substitute markers for clinical/scan assessment	Follow the individual bone-health and disease plan
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00776 v3b · official regimen · EMA Enhertu product information · retrieved 25 October 2026 · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · ESO-ESMO ABC 5 consensus: tumour-marker interpretation

THE WHOLE TREATMENT JOURNEY

Trastuzumab deruxtecan (Enhertu) · every 3 weeks: treatment and checks

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Unresectable or metastatic HER2-positive or HER2-low breast cancer, within the NCCP prior-treatment criteria

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP 00776	Confirm exact indication and biomarkers	Usual starting dose 5.4 mg/kg intravenously on Day 1 every 21 days; prescribed reductions are 4.4 then 3.2 mg/kg	One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.	Continue with safety / benefit review	Treatment continues while it controls the cancer and side effects remain acceptable. This advanced-disease regimen has no automatic one-year completion date. A good scan does not by itself instruct a treatment break.
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or radiotherapy (RT) implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP 00776 · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1



Blood tests

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed. Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan.

Checks safety before continuation.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

2

Heart checks

NCCP specifies echo/MUGA before treatment and every 12 weeks. With uninterrupted three-week cycles, week 12 is approximately before cycle 5 and week 24 before cycle 9. Delays do not reset the elapsed-time monitoring clock. Signs of cardiac impairment can require 4–8-weekly checks; an abnormal result meeting the treatment-hold criteria requires repeat assessment within three weeks.

Checks cardiac safety when the agent or clinical risk requires it.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

3



Imaging

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Uses the correct surveillance or response-assessment pathway.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

4

Bone health

Enhertu alone does not require a routine bone-density scan schedule. DEXA, bone medicines or bone scans are added for another treatment, fracture risk, symptoms or disease assessment when indicated.

Companion endocrine therapy and individual fracture risk may change the plan.

Ask / act: Agree the due window, booking team and result reviewer. Contact the team if a test is overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Blood tests assess treatment safety. Scans assess disease or investigate symptoms. Echo and electrocardiogram (ECG) answer different questions.

Before treatment

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed.

During treatment

Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan.

Heart checks

NCCP specifies echo/MUGA before treatment and every 12 weeks. With uninterrupted three-week cycles, week 12 is approximately before cycle 5 and week 24 before cycle 9. Delays do not reset the elapsed-time monitoring clock. Signs of cardiac impairment can require 4–8-weekly checks; an abnormal result meeting the treatment-hold criteria requires repeat assessment within three weeks.

Bone health

Enhertu alone does not require a routine bone-density scan schedule. DEXA, bone medicines or bone scans are added for another treatment, fracture risk, symptoms or disease assessment when indicated.

Your wider treatment plan

Additional treatments depend on the confirmed setting and indication.

Advanced-disease scans

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Tumour markers

If informative, particularly when elevated at baseline or disease is not readily measurable, tumour markers may supplement clinical assessment and imaging. A marker rise alone should not trigger a treatment change. There is no universal fixed marker timetable.

Pregnancy, fertility and contraception

Where relevant, use effective contraception during treatment and for at least seven months after the last dose for women, or four months for men with a partner who could become pregnant. Do not breastfeed during treatment and for seven months after the last dose. Discuss fertility before starting.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Enhertu infusion	 **Day 1** Day 1 infusion; next dose normally three weeks later.
Nausea may be delayed	 **Days 1–21** Symptoms vary and can continue after infusion day. Use the prescribed prevention/rescue plan; this band is not a prediction of daily symptoms.
Energy and blood counts	 **Days 1–21** Fatigue and low blood counts can affect any part of the cycle. Bloods and symptoms guide the next dose; no guaranteed recovery day is shown.
Lung symptoms: report promptly	 **Days 1–21** New cough, breathlessness or fever needs urgent contact on any day, including after earlier well-tolerated cycles.

Treatment and rest: One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

Tests and appointments: Use the monitoring table for baseline and later checks. A rest week does not remove infection or other risks.

> **Why these bands stay broad:** Treatment days are specified by the regimen. Symptom timing varies; the full-cycle symptom bands show that an effect can occur at any point, not a measured probability or severity. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00776 v3b · official regimen ↗ · EMA Enhertu product information · retrieved 25 October 2026 ↗

There are no guaranteed "safe days". Contact the oncology team urgently for any new or worsening cough, breathlessness or fever: lung inflammation can start at any point and can become serious. Do not wait until the next infusion or scan. Also report fever or feeling acutely unwell, unusual bleeding, inability to keep fluids down, chest pain or new ankle swelling. Severe breathing difficulty or collapse needs emergency care.

Evidence depends on the population

Different studies answer different questions; do not compare their results as if populations and follow-up were identical.

DESTINY-Breast04: previously treated HER2-low advanced breast cancer

557 patients with HER2-low metastatic breast cancer after one or two previous chemotherapy lines; 494 had hormone-receptor-positive disease. T-DXd versus physician's-choice chemotherapy. Blinded-central-review PFS in the hormone-receptor-positive cohort; PFS and OS in all patients were key secondary endpoints. In the HR-positive cohort, median PFS was 10.1 versus 5.4 months (HR 0.51). In all patients, median PFS was 9.9 versus 5.1 months (HR 0.50) and median OS 23.4 versus 16.8 months (HR 0.64). These are the original 2022 analysis, also reproduced in EMA SmPC §5.1. The HR-negative subgroup was much smaller. The trial does not establish benefit for HER2 IHC0 tumours or every earlier treatment setting. Drug-related ILD/pneumonitis occurred, including fatal cases; benefit must be discussed alongside pulmonary risk.

DESTINY-Breast04: previously treated HER2-low advanced breast cancer

Practical support

Prevent nausea before it starts

This regimen has high emetic risk. An antiemetic combination is prescribed before every infusion, with an agreed plan for delayed symptoms and breakthrough nausea. The dedicated NCCP trastuzumab-deruxtecan pathway should be checked by the prescriber and pharmacist.

Lung-safety plan

Carry the patient alert card and report new cough, fever or breathlessness promptly. Steroids used for suspected lung inflammation are prescribed and monitored by the clinical team; do not start them yourself from this information.

Blood-count support

Low blood counts can cause infection, anaemia or bleeding. The need for granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or transfusion support depends on results, symptoms and previous complications; it is not automatic for every cycle.

Eating and energy

Small frequent meals and early treatment of nausea can help maintain intake. Tell the team about weight loss, constipation or diarrhoea so that support can be tailored.

Taking tablets / attending treatment

Practical treatment details

Before each visit

Bring your medication list.

On infusion day

Tell the nurse immediately about flushing, chills, rash, dizziness, wheeze or breathing difficulty during treatment.

Between infusions

Drink regularly.

Keep activity manageable and ask for help with fatigue.

If you already have a port

A trained nurse accesses and removes the port needle for the infusion visit, then flushes the line according to local policy.

Report redness, pain, swelling or fever.

Questions to ask

When do I take or receive treatment?

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses. The first infusion takes 90 minutes. Later infusions may take 30 minutes if the previous infusion was tolerated. Allow extra time for assessment, anti-sickness medicines and any observation. A cannula or an existing central line can be used according to the clinical plan; there is no home pump.

When are bloods, ECGs or echoes needed?

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed. Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan. NCCP specifies echo/MUGA before treatment and every 12 weeks. With uninterrupted three-week cycles, week 12 is approximately before cycle 5 and week 24 before cycle 9. Delays do not reset the elapsed-time monitoring clock. Signs of cardiac impairment can require 4–8-weekly checks; an abnormal result meeting the treatment-hold criteria requires repeat assessment within three weeks.

Explain the plan back: Which medicine is taken or given when, what checks are expected next, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Trastuzumab deruxtecan (Enhertu) · every 3 weeks

Active medicines: Trastuzumab deruxtecan (Enhertu)

Emetogenic risk: High · dedicated trastuzumab deruxtecan row

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before treatment
Ondansetron	16 mg orally	Day 1 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 1 before treatment Reconcile all steroid premedication; do not duplicate.
Aprepitant	80 mg orally once daily	Days 2–3
Dexamethasone	8 mg orally once daily	Days 2–4 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This regimen has high emetic risk. An antiemetic combination is prescribed before every infusion, with an agreed plan for delayed symptoms and breakthrough nausea. The dedicated NCCP trastuzumab-deruxtecan pathway should be checked by the prescriber and pharmacist.
- Carry the patient alert card and report new cough, fever or breathlessness promptly. Steroids used for suspected lung inflammation are prescribed and monitored by the clinical team; do not start them yourself from this information.
- Low blood counts can cause infection, anaemia or bleeding. The need for granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or transfusion support depends on results, symptoms and previous complications; it is not automatic for every cycle.
- Small frequent meals and early treatment of nausea can help maintain intake. Tell the team about weight loss, constipation or diarrhoea so that support can be tailored.
- NCCP NCIS v8 Table1 has a dedicated trastuzumab-deruxtecan row: NK1 + 5-HT3 + dexamethasone. Olanzapine can be considered by the prescriber, but is not silently added to that national default row.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

HER2-directed antibody–drug conjugate · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 New cough or breathlessness

trastuzumab_deruxtecan

Possible lung inflammation needs urgent assessment at any time; do not wait for a scan appointment.

3 Nausea and intake

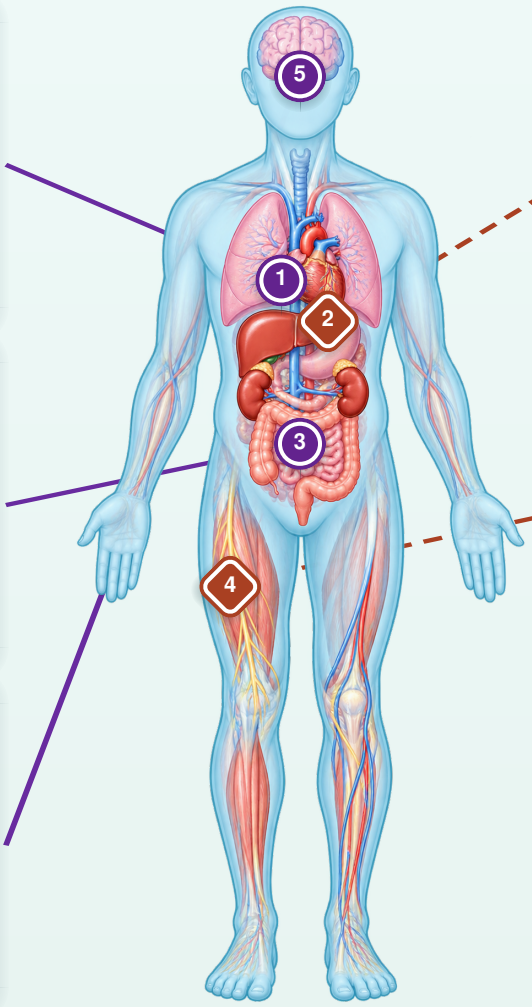
trastuzumab_deruxtecan

Nausea can be delayed. Use the prescribed prevention and rescue plan; call if fluids will not stay down.

5 Hair loss

trastuzumab_deruxtecan

Hair thinning or loss can occur. Ask about scalp and practical support before treatment starts.



2 Heart pumping function

trastuzumab_deruxtecan

Keep baseline and 12-weekly echo/ multigated acquisition (heart pumping) scan (MUGA) appointments. Report breathlessness, chest symptoms or new swelling.

4 Blood counts / infection

trastuzumab_deruxtecan

Bloods are checked before each cycle. Fever, feeling suddenly unwell or unusual bleeding needs prompt contact.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for any new or worsening cough, breathlessness or fever: lung inflammation can start at any point and can become serious. Do not wait until the next infusion or scan. Also report fever or feeling acutely unwell, unusual bleeding, inability to keep fluids down, chest pain or new ankle swelling. Severe breathing difficulty or collapse needs emergency care. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00776 v3b · official regimen ↗**
   https://healthservice.hse.ie/documents/6661/Trastuzumab_776_Deruxtecan_5.4mg_Therapy.pdf
2. **EMA Enhertu product information · retrieved 25 October 2026 ↗**
   https://www.ema.europa.eu/en/documents/product-information/enhertu-epar-product-information_en.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
6. **Cortés et al. Nature Medicine2024 · DESTINY-Breast03 long-term analysis**
   https://pubmed.ncbi.nlm.nih.gov/38825627/
7. **DESTINY-Breast04: previously treated HER2-low advanced breast cancer**
   https://pubmed.ncbi.nlm.nih.gov/35665782/
8. **DESTINY-Breast03 long-term analysis (2024)**
   https://www.nature.com/articles/s41591-024-03021-7
9. **DESTINY-Breast04 long-term analysis (2025)**
   https://www.nature.com/articles/s41591-025-03981-4
10. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf